



For Staff

Self-Harm Indicators

Responding calmly when you notice signs that a student may be harming themselves: dignity, first aid, asking about suicide, and getting the right help

Content Warning This guide covers self-harm and suicidal thoughts.

This is reference guidance, not a training course. Use it to check how to respond, or to prepare for a conversation. It does not replace the school's safeguarding training, policy or the DSL's advice on a specific case.

It contains discussion of self-harm, injuries and suicidal thoughts, without any description of methods. Some of it may be difficult to read, particularly if it connects with your own experience or that of someone close to you.

- You can read it in sections, take a break, or step away at any point.
- If this raises something personal, you can speak in confidence to your line manager, a senior colleague you trust, or the school's Employee Assistance Programme, if it has one, or **Education Support** (free, confidential, 24/7 on 08000 562 561). **Samaritans** is free, any time, on 116 123.
- If you have a question about how this guidance applies to a situation you are dealing with, speak to your school's Designated Safeguarding Lead (DSL).

If reading this makes you worried about a child now, do not wait until you have finished reading: speak to the DSL or a deputy straight away. If a child is in immediate danger, call 999.

Immediate danger or possible risk to life: call 999 when it is needed, stay with the child, and summon the DSL or a deputy. Don't delay action to work through the steps. The 90-second Pause is for once immediate safety is secured, to stop an impulsive or investigative response. **Go to the Quick Reference Card.**

Why This Matters

Self-harm is common among teenagers, and most of it is hidden. In a UK study of more than 10,000 17-year-olds, **24.1% had self-harmed in the previous 12 months** ([Patalay and Fitzsimons, 2021](#)). Self-harm is usually a way of coping with overwhelming feelings, not attention-seeking. It is also a signal that matters: **around half of young people under 25 who died by suicide had previously self-harmed** ([NCISH, 2017](#)).

KCSIE 2026 says all staff should be aware that mental health problems can, in some cases, develop into safeguarding concerns, including self-harm (paragraph 47), and lists physical signs of self-harm among the warning signs staff are well placed to recognise (paragraph 48).

Scenario Overview

While collecting art projects, you notice fresh cut marks on a Year 9 student's forearm when their sleeve rides up. They pull the sleeve down quickly and don't meet your eye.

What to Keep in Mind

- **Respond calmly and privately.** Shock, disgust or a public comment can deepen shame and make the student hide it more.
- **Fresh injuries need first aid.** Get the school nurse or a first aider. Call 999 for a serious injury.
- **Ask about suicide as well.** Self-harm and suicidal thoughts are different, but they can go together. Asking directly does not put the idea in someone's head (Samaritans).
- **No disclosure is needed.** The student may deny it or give another explanation. Tell the DSL the same day anyway.
- Self-harm is often a **pattern**. A pattern can help explain what is happening, but **you don't need to wait for it to happen again** before telling the DSL.

How RESPOND Works in This Scenario

RESPOND is powered by **ACT: Active Intervention** (inaction is not neutral), **Contextual Safeguarding** (friends, online spaces and pressures at school and home all play a part) and **Trauma-Informed Practice** (safety before emotion before thinking). The steps are taught in order but work as a **helix**: see *The RESPOND Helix* section.

Step	RESPOND tool	In this scenario
● Recognise	4 Ps Context Scan; KCSIE warning signs	Physical signs, and the changes around them
● Engage	TED questions; ask directly about suicide	A private, calm check-in; no demand to see injuries
● Support	Safety check first; "Your calm is contagious"	First aid for fresh injuries; 999 if serious
● Pause	90-second grounding; Three Lenses	Don't investigate, confront or promise secrecy
● Offer	Model alternatives; if in doubt, N before O	Other ways to cope, and people who can help
● Notify	DSL the same day; immediately where there is risk	The DSL leads on health routes and parents
● Document	GOLD standard	Plain description of what you saw; their exact words

RESPOND is not a checklist to complete in order. Immediate danger or possible risk to life: act first (999 when needed, stay with the child, summon the DSL or a deputy), then return to the steps.

Need the essentials? The *Quick Reference Card* at the end of this guide covers the immediate response and escalation on one page.

The One Chance Rule: a disclosure may be a young person's only attempt to seek help. Make it count.



The RESPOND Helix: How the Steps Move

RESPOND is taught in a line, Recognise to Document, because sequence gives structure under pressure. In practice it moves as a helix. The first dimension is the steps themselves: you may loop back, hold at a step, or go straight to Notify. The second is time and severity: each turn sits higher as a concern develops. The third is context: a concern involving several children, places or platforms traces a wider spiral.

Relational and Systemic

For a child, safeguarding is **relational**: it happens through people they trust. For the adults around them, it is also **systemic**: the steps, records and shared decisions that make sure what a child tells one person reaches the people who can help.

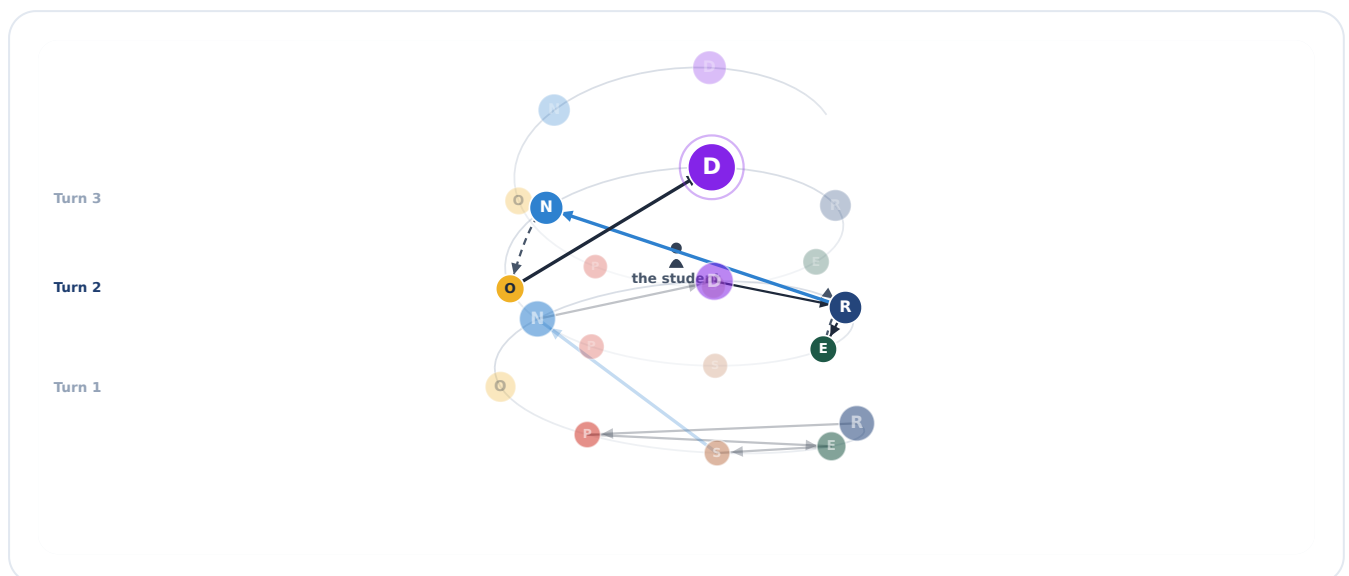
Relational is why children tell. Systemic is why telling works.

The RESPOND steps follow the same shape: Recognise, Engage and Support are relational; Pause is the bridge; Offer, Notify and Document are the systems steps. Neither works alone. Relationship without the system becomes secrecy, which is why staff never promise to keep a disclosure to themselves. The system without relationship becomes process that children do not trust enough to use. When you share information, be open with the child about what you are sharing and why.

See It Move

One example: injuries noticed in a lesson, which lead to a disclosure weeks later. Then see what happens when steps are skipped.

This is a still image of the interactive RESPOND Helix. Open the web version of this guide to step through the examples; every example is also written out in full below.



▼ Read the helix demonstration as text

Sequence

How RESPOND is taught.

1. **The seven steps:** RESPOND is taught in this order, Recognise to Document, because sequence gives structure under pressure. Recognise, Engage and Support are the relational steps; Pause is the bridge; Offer, Notify and Document are the systems steps.
2. **But practice is not a line:** Self-harm is often hidden, and it may be noticed long before it is talked about. You may loop back, hold at a step, or go straight to Notify. Choose an example to see how the steps move.

Noticed, not told

A Year 9 student whose injuries are noticed in an art lesson, and who talks weeks later.

1. **Turn 1: a sleeve rides up:** Collecting art projects, the teacher sees fresh cuts on a Year 9 student's forearm. The student pulls the sleeve down quickly. Recognise: no disclosure is needed.

2. **Turn 1:** Pause: a breath. Don't react with shock in front of the class, and don't ask the student to show you. Protect their dignity.
3. **Turn 1:** Engage: at the end of the lesson, privately: "I noticed your arm earlier, and I'm a bit worried about you. How are you doing?" The student says it was "the cat".
4. **Turn 1:** Support: "OK. I'd like the nurse to take a look, just to check it's clean." The teacher walks with them. Fresh injuries get first aid.
5. **Turn 1:** Notify: the DSL, the same day. You don't need the student to confirm it is self-harm, and you don't need to wait for it to happen again.
6. **Turn 1:** Document: GOLD. What you saw, described plainly, and the student's exact words.
7. **Turn 2: three weeks later:** The student lingers after the lesson and says: "The cat thing wasn't true. I do it when everything gets too much."
8. **Turn 2:** Engage: thank them, and use the script: "I believe you, and I'm taking what you've told me seriously. This is not your fault. I'm going to help keep you safe, and I'll explain what happens next." Then TED, and ask directly: "Do you ever have thoughts of ending your life?"
9. **Turn 2: loop back:** Loop back to Recognise: the student says they sometimes wish they could disappear. Self-harm and suicidal thoughts can go together; this changes the level of concern.
10. **Turn 2: N before O:** Notify before Offer: the DSL, immediately, while the student stays with you. The DSL decides on mental health advice and how parents are told.
1. **Turn 2:** Offer, once the DSL has decided: model alternatives. What helps when feelings get too much, who to go to, and a plan made with the student.
2. **Turn 2:** Document: GOLD, with their exact words, the question you asked, and their answer.

When it breaks

The same Year 9 student, when steps are skipped.

1. **Turn 1:** The teacher sees fresh cuts on the student's forearm. Recognise happens.
2. **Pause skipped:** The teacher says, in front of others: "What have you done to your arm? Roll your sleeve up." The student is humiliated, and hides it more carefully from then on.
3. **Document skipped:** Nothing is recorded. When a friend later raises a concern, there is nothing to connect it to.
4. **Notify skipped:** The teacher decides it is "probably attention-seeking" and tells no one. The DSL never hears, and no one asks the student about suicide.
5. **The helix fragments:** The student has learned that being seen brings shame, not help. That is how harm escalates unseen.
6. **Put it back together:** Every step taken puts the structure back. Stay calm, check in privately, tell the DSL. Relational is why young people tell; systemic is why telling works.

The Helix is described in the RESPOND Safeguarding Operating System (section 5.2). All examples are fictional.



RECOGNISE: Notice Without Judging

Self-harm is when someone hurts themselves on purpose, usually as a way of coping with feelings that are too much. You don't need to be sure. If you notice signs, the DSL needs to know.

The 4 Ps Context Scan

People

Friendships changing, a relationship ending, bullying, loss, pressures at home, friends who seem worried or who self-harm too

Places

PE changing rooms, swimming, art and practical lessons, the toilets, the boarding house, trips and residentials

Platforms

Posts, messages or searches about self-harm; content they have seen or shared; online communities

Patterns

Repeated injuries or excuses, covering up in warm weather, avoiding PE, changes in mood, sleep or eating

Signs You Might Notice

Physical

- Unexplained cuts, burns, bruises or marks, often on arms, wrists, thighs or stomach
- Blood on clothes, or plasters and dressings that keep appearing
- Long sleeves or layers in hot weather; refusing to change for PE or swimming

Emotional and Behavioural

- Withdrawing from friends or activities they used to enjoy
- Low mood, irritability, tearfulness, or seeming numb

- Ongoing difficulty sleeping; tiredness in lessons
- Saying they are worthless, or that they deserve to be hurt

KCSIE 2026 lists significant changes in behaviour, ongoing difficulty sleeping, withdrawing from social situations, not wanting to do things they usually like, and physical signs of self-harm or neglecting themselves as potential warning signs (paragraph 48). Not every student with these signs is self-harming (paragraph 49), but each is a reason to check in.

No Disclosure Needed

The student may deny it, blame a pet or an accident, or say it's nothing. Don't argue, and don't try to prove it. Pass what you saw to the DSL **the same day**, and record it factually. You don't need to wait for it to happen again.

What Self-Harm Is, and Isn't

- It is usually a way of **coping** with overwhelming feelings, such as distress, anger, numbness, shame or feeling out of control. It can bring short-term relief, which is part of why it can continue.
- It is **not attention-seeking**. Most self-harm is hidden. If a student does want attention, it is because they need help.
- It is **not the same as a suicide attempt**, but it is linked: around half of young people under 25 who died by suicide had previously self-harmed (NCISH, 2017). Always ask about suicide.
- It can be an indicator that a child has suffered, or is at risk of, abuse, neglect or exploitation (KCSIE 2026, paragraph 47).

E

ENGAGE: A Calm, Private Check-In

Choose your moment. Speak privately, calmly and kindly, and focus on how the student is, not on the injury. Use TED, never "why".

Opening the Conversation

Don't comment in front of the class. At the end of the lesson, or at a quieter moment soon after, ask the student to stay behind or walk with you. If the injuries look fresh or need care, don't wait: go with them to the nurse.

"I noticed your arm earlier, and I wanted to check how you're doing. You're not in trouble."

"Sometimes when people are going through a hard time, they hurt themselves to cope. Is that something that's been happening for you?"

What a Student Might Say

"It's nothing. It was the cat."

"It helps. It's the only thing that makes it stop."

If they deny it, don't push or argue: "OK. I'm glad you're OK. If that changes, or you want to talk, I'm here." You still tell the DSL.

When They Tell You

The **RESPOND** disclosure script:

"I believe you, and I'm taking what you've told me seriously. This is not your fault. I'm going to help keep you safe, and I'll explain what happens next."

- **TED:** "Tell me about what's been happening." "Explain what things are like when it gets too much." "Describe what helps, even a little."
- **Ask directly about suicide:** "Sometimes people who hurt themselves also have thoughts of ending their life. Is that something you've been thinking about?"
- **Check today:** "Have you hurt yourself today? Does anything need looking at?"
- "Who else knows?"
- You don't need to know how they hurt themselves, and you shouldn't ask for details.

Helpful

- "Thank you for telling me. That can't have been easy."
- "It sounds like you've been dealing with a lot on your own."
- "You're not in trouble, and you're not on your own with this."

Unhelpful

- "Why would you do that to yourself?" ("why" sounds like blame)
- "Show me your arm." "Roll your sleeve up." (humiliating, and not your role)
- "You need to promise me you'll stop." (may lead them to hide it)
- "Are you just doing it for attention?" (shaming, and untrue)

If Other Students Notice

If it is safe to do so, protect the student: **Distract** (move the lesson on), **Direct** ("That's enough, back to work"), **Delegate** (a colleague takes the group) or **Delay** (speak to those students afterwards). Tell the DSL who saw.

S

SUPPORT: Care First

Physical care and emotional safety come first. Your calm is contagious: a matter-of-fact, kind response tells the student it is safe to accept help.

Injuries and Medical Care

- **Fresh or open wounds:** go with the student to the school nurse or a first aider, using the school's first aid procedures. Don't send them alone.
- **Call 999** for a serious injury, if the bleeding won't stop, if the student has taken an overdose or swallowed something harmful (even if they seem well), or if they cannot keep themselves safe.
- NICE NG225 says professionals in non-health settings should address immediate physical health needs from self-harm, in line with local policies, and call 111 or 999 if necessary (recommendation 1.8.1).
- Let the nurse or first aider assess injuries in private. Wear gloves, as for any wound.

If There Are Suicidal Thoughts or Immediate Risk

If the student says they have thoughts of ending their life, has hurt themselves seriously, or you are unsure whether they are safe, **don't leave them alone**. Stay with them and send for the DSL or a deputy now. See Staff Guide 34 (Student Expresses Suicidal Thoughts) for the full response.

Emotional Support

- A quiet, private space, and time. Don't send them straight back to a lesson if they are upset.
- "You're not in trouble. I'm glad I know, because now we can help."
- "You don't have to stop on your own right now. Let's get you some support."

SEND, Neurodivergence and Language

- Allow extra processing time. Use clear, concrete language and one question at a time.
- Offer other ways to communicate: writing, drawing, a feelings scale, or an adult who knows them well.
- Use a professional interpreter where language is a barrier, never another student or a family member.
- Record any adjustments you made.

Practical Adjustments to Discuss With the DSL

- Changing for PE or swimming privately, or a discreet alternative
- Uniform flexibility, for example long sleeves, without drawing attention
- A discreet way to leave a lesson and a safe place to go
- A named key adult and regular check-ins
- Don't make these on your own initiative: the DSL and the student agree them.

Friends Need Support Too

Friends who know may be frightened, carrying a secret, or self-harming themselves. Tell them they did the right thing by telling an adult, that it is not their job to keep their friend safe, and that they are not in trouble. Pass their names to the DSL.

P PAUSE: Consider Your Approach

Act with composure. Take 90 seconds: breathe, lower your shoulders, and check your own reaction before you speak. If there is immediate risk, the Pause is a breath while you act, not a delay.

Three Lenses

Lens	In this scenario
Risk	Are the injuries fresh or serious? Has the student had thoughts of suicide? Have they taken anything? Is anything happening at home or online that could be behind this?
Rights	The student's right to dignity and privacy, to be listened to without judgement, and to a say in what happens next and how their parents are told

Resources The DSL and deputies, the school nurse, the mental health lead and counsellor, NHS 111 (mental health option), 999, parents and carers, and trusted adults in the student's life

What You Must Not Do

Don't Investigate

- Don't ask to see injuries, or ask the student to roll up sleeves or remove clothing
- Don't search their bag, room or phone, or ask friends to check on them
- Don't ask how they hurt themselves, or with what

Don't Promise Secrecy

"I can hear you're worried about who will find out. I can't keep this to myself, because I care about your safety. I'll speak to the DSL, whose job is to help, and we'll talk with you about how your parents are told."

Don't Punish, Shame or Make It Public

- Self-harm is not a behaviour issue: no sanctions
- Don't call it attention-seeking, or insist they promise to stop
- No comments in front of others, and don't discuss it with other students

Check Your Own Response

- Notice your own reaction: shock, fear, sadness, or the urge to make it stop. All are normal. Your calm is contagious.
- If you feel overwhelmed: "This is important and I want to get it right. Let's go and see the nurse together, and I'll speak to the DSL."



OFFER: Model Alternatives

Offer hope, choices and other ways to cope, without pressure, and explain what happens next. If in doubt, N before O: tell the DSL before offering options.

Explain What Happens Next

"Here's what happens now. I'm going to talk to our safeguarding lead, whose job is to make sure you get the right support. The nurse will check your arm. Because you're under 18 and this is about your safety, your parents or carers will usually need to know; we'll talk with you about how that happens. You're not in trouble. What questions do you have?"

Be honest about information sharing. Staff should share safeguarding information as early as possible, and data protection law does not prevent it; safeguarding children is a lawful basis for sharing sensitive information without consent where appropriate (KCSIE 2026, paragraphs 142 to 148). What is shared must be relevant, necessary and proportionate, go only to those who need it, and be handled in line with data protection law. From 30 September 2026, the Information Sharing Duty (section 16LA, Children Act 2004) also requires the organisations it covers to share where its tests are met; the DSL decides how it applies. So don't ask for consent as if it were the legal basis. Tell the student what will be shared, with whom and why, unless that would increase the risk, and listen to their views: they shape the plan. Agreeing to a service, such as counselling, is a separate question, and their agreement matters there.

Options, Not Orders

- "Would you like me to come with you to the nurse?"
- "How would you like your parents to be told: with you there, or with us leading?"

Some things have to happen to keep them safe. Where there is a choice, give it to them.

Model the Alternative

What You Can Model

- Feelings can be talked about, and they can change
- There are other ways to get through the worst moments, and support to find them
- Asking for help is a strength, not a failure

Don't ask the student to give up self-harm straight away; specialist support helps with that.

Support in School and Beyond

- The school nurse, counsellor or mental health lead; a regular check-in with a trusted adult
- A support plan developed with the student and, where appropriate, their family and mental health professionals
- Childline (for anyone under 19): 0800 1111, free and confidential; call any time, day or night, or use 1-2-1 chat at [childline.org.uk](https://www.childline.org.uk) (you may have to wait for a chat counsellor, so calling is usually quickest)
- Shout: text **SHOUT** to 85258, free, confidential, 24/7
- Samaritans: 116 123, free from any phone, any time, day or night, whatever your age
- NHS 111: call 111 and choose the mental health option (option 2), for urgent mental health help in England, any time, or use NHS 111 online
- Stay Alive app (Grassroots Suicide Prevention): free, with a personal safety plan and a directory of help

N NOTIFY: Tell the DSL Today

Tell the DSL as soon as is practicably possible, and the same day, even if the student denied it. Where there is immediate risk (a serious injury, anything taken, or thoughts of suicide), tell the DSL immediately and don't leave the student alone. If a life is at risk, call 999.

Who and When

- **Signs noticed, no immediate risk:** the DSL, as soon as is practicably possible, and before the student goes home.
- **Fresh injuries:** first aid and the nurse now, then the DSL.
- **Suicidal thoughts, a serious injury, or anything taken:** the DSL immediately, with the student kept safe and not alone. Life at risk: 999 first.
- If the DSL and deputies cannot be reached, go to a senior leader. Anyone can refer directly to children's social care if they believe a child is at risk; telling the DSL does not remove your own responsibility.

What to Tell the DSL

- What you saw, described plainly (for example, "several fresh cuts on the left forearm"), and when
- Exactly what the student said, in their words, including any explanation or denial
- Any first aid or medical care given, and by whom
- Who else knows, including friends, and anything online
- What you have said and done, including anything you said would happen next

▼ For the DSL: assessment, external agencies and parents

Most staff need only: tell the DSL, today, with the information above.

The DSL's Initial Actions

- Make sure physical health needs are met and the student is safe today
- Speak with the student, including asking directly about suicide, and seek mental health advice where needed: NHS 111 (mental health option), or 999 and A&E where the student is in danger (KCSIE 2026, paragraph 50)
- Decide who contacts parents or carers, when and how, considering whether this could place the student at additional risk (KCSIE 2026, paragraph 229)
- Consider referral to the GP and local children and young people's mental health services, with the student and their parents; involve the MHST where the school has one
- Develop a support plan with the student, their family and mental health professionals (NICE NG225, recommendation 1.8.6), including practical adjustments at school. Don't rely on risk assessment tools or scores to predict risk (recommendation 1.6.1)
- Consider abuse, neglect or exploitation behind the self-harm (KCSIE 2026, paragraph 47); support friends who know (NICE NG225, 1.8.7)
- Apply the Information Sharing Duty: share what is relevant and proportionate with those who need it; record the decision to share or not share, and be transparent with the student
- Record on the safeguarding system, with a chronology, and review

Routes to Help

Route	When
999 (ambulance)	A life is at risk: serious injury, an overdose or anything harmful swallowed (even if they seem well), or a student who cannot keep themselves safe

Route	When
A&E	A child is in danger: call 999 or arrange for them to be taken to A&E immediately by a parent, carer or other suitable person (KCSIE 2026, paragraph 50)
NHS 111, mental health option	Urgent mental health help that is not an emergency: call 111 and choose the mental health option (option 2), any time, or use NHS 111 online (KCSIE 2026, paragraph 50)
Local children and young people's mental health services (CYPMHS, often still called CAMHS)	Assessment and treatment; referral by the GP, school or another professional (self-referral in some areas)
GP	Care of injuries, review and referral; usually arranged with parents
Mental Health Support Team (MHST)	Early support and advice, where the school has access (KCSIE 2026, paragraph 232)
Children's social care	Signs of abuse, neglect or exploitation behind the distress (KCSIE 2026, paragraph 47); where telling parents could increase the risk; or where parents cannot keep the child safe
Police	A student who is missing and at risk, or immediate danger to anyone

Where a Student Is 18

At 18 a student is legally an adult. The child safeguarding framework in KCSIE applies to under-18s, but the school still has welfare responsibilities. Where an 18-year-old is involved, the DSL considers adult safeguarding, police and welfare pathways alongside the school's procedures, and the adult student's own views and consent in a different way. This matters in sixth form and in boarding, where students turn 18 during their final year.

Parents and Carers: Usually Yes, but the DSL Decides

Parents and carers are usually a key part of keeping a child safe, and they need to know when there is risk. KCSIE 2026 says the DSL should consider whether to inform parents or carers to support the child's wellbeing, **where doing so would not place the child at additional risk** (paragraph 229). Where the student says something at home is part of the reason, where there are signs of abuse or neglect, or where honour-based abuse is possible, the DSL seeks advice from children's social care before contacting parents and agrees a safe plan. Tell the student honestly that parents will usually need to know, and ask how they would like it done. Don't promise that parents won't be told.

Parent contact: what to say

- "I'm calling because a member of staff noticed some injuries on [name]'s arm today, and [name] has told us they have been hurting themselves when things feel too much. The nurse has checked them."
- "[Name] is not in trouble. Please stay calm with them and listen; anger or panic can make it harder for them to talk."
- "Here is what we recommend next, and the numbers if you're worried tonight: NHS 111 option 2, Samaritans on 116 123, and 999 in an emergency."

D DOCUMENT: Record Observations

Record as soon as is practicably possible, on the school's safeguarding system, using **GOLD**. Describe what you saw plainly and briefly; don't speculate.

GOLD

Ground

Date, time, place, who was present, who you are, and how it came to your attention

Observe

What you saw and heard, with exact words in quotation marks

Language

Neutral, factual and non-diagnostic. No opinions or labels

Do

What you did, what you said would happen next, and why

Example: What You Saw and Exact Words

- Observed: when the student's left sleeve rode up, several short, fresh-looking cuts on the inner forearm; student pulled the sleeve down straight away
- Student said: "It's nothing. It was the cat."
- Later, student said: "I do it when everything gets too much."
- When asked about thoughts of ending their life, student said: "No, it's not like that."
- Actions: walked with the student to the school nurse at 11:40am; told the DSL in person at 11:55am

Recording template

SAFEGUARDING CONCERN: SIGNS OF SELF-HARM

Date and time:

Student: name, year, tutor group

Recorded by: name and role

Where and how it came to your attention:

WHAT I SAW (plain description, location on body, fresh or healed; no method detail):

WHAT THE STUDENT SAID (exact words in quotation marks, including any explanation or denial):

QUESTION ABOUT SUICIDE AND THEIR ANSWER (exact words):

FIRST AID OR MEDICAL CARE (by whom, time):

WHO ELSE KNOWS (friends, family, online):

WHAT I DID AND SAID (with times):

SHARED WITH: DSL (name, time, how)

THE STUDENT'S WISHES AND VIEWS:

Signed and dated:

What Not to Record

- Opinions or labels such as "attention-seeking" or "dramatic"
- A diagnosis, or guesses about causes or the family
- Detail about how the injuries were caused, beyond what the student said and what you saw
- Photographs of injuries: never take them

Boarding and Residential Life

For boarders, school is also home. House staff are often best placed to notice, and self-harm may happen more in the evening or at night.

Step	In boarding
● Recognise	Use the 4 Ps for the house: showers and changing, room-mates, evenings and weekends, phones after lights out. House staff may notice injuries, blood on bedding or towels, or long periods alone.
● Engage	Check in privately and calmly. Don't ask to see injuries. Ask directly about suicide. Talk in a visible space with another adult aware.

Step	In boarding
● Support	Fresh injuries: the health centre or on-call nurse, or 999 if serious. Safety tonight: who is on duty, and how does the student reach them?
● Pause	Don't search rooms, investigate or promise secrecy. If there may be immediate risk, don't wait until morning.
● Offer	Model alternatives: evening routines and company, and a clear way to find staff at night. If in doubt, N before O.
● Notify	The on-call DSL immediately where there may be immediate risk, including at night; otherwise the DSL as soon as is practicably possible. Life at risk: 999.
● Document	Record on the safeguarding system before you go off duty, using GOLD. A house log or handover note is not a substitute.

Boarders' Routes to Help

- **Boarders can always contact their house parents, the DSL or a member of the boarding management team, at any time, day or night.** Make sure every boarder knows how.
- An independent person outside the staff whom boarders can contact directly (NMS 11.4), often called the independent listener
- Boarders must be able to contact their families privately; monitoring of communications should be proportionate (NMS 12.1)
- International boarders: agree with the DSL who contacts parents or guardians, when, and in what language, using interpreters where needed

Source: National Minimum Standards for Boarding Schools (2026).

Night-time arrangements, proportionate checks and how access to anything that could be used to cause harm is managed are set out in Staff Guide 34, *Boarding and Residential Life*. The DSL and health team decide these; house staff don't search rooms.

Shared With Scenario 34

Scenarios 7 and 34 share several sections. They are summarised here; the full versions are in **Staff Guide 34: Student Expresses Suicidal Thoughts**.

Return After a Crisis	The DSL or mental health lead plans any return with the student, their family and health professionals: a key adult, a discreet exit from lessons, adjustments and a review date.
Safety Plans	Made with the student (and family where appropriate): warning signs, what helps, who to contact. Health professionals lead where involved; don't rely on risk scores to predict risk (NICE NG225, 1.6.1, 1.11.7 and 1.11.8).
Peers, Online and Contagion Risk	Ask who else knows; the DSL decides who speaks to friends. Don't look through phones. Don't discuss details in class. Use safe language: never "attention-seeking", no method detail, always point to help. Samaritans Step by Step supports schools after a suspected or attempted suicide.
Identity and Intersecting Risk	LGBTQ+, neurodivergent, care-experienced, bereaved and international students may face extra pressures (Patalay and Fitzsimons, 2021; NCMD, 2021). Notice and ask; never assume, and never share a student's identity without the DSL's advice.
Looking After Yourself	Seeing injuries or hearing a disclosure can be distressing. Ask the DSL for a debrief, and use the Employee Assistance Programme if the school has one, or Education Support.
Additional Resources	Contacts for students, parents and staff, statutory guidance, and a notice that Papyrus HOPELINE247 closed in September 2026: replace the old number wherever it appears.

Guidance

- Department for Education, *Keeping Children Safe in Education 2026*
- NICE, *Self-harm: assessment, management and preventing recurrence* (NG225), recommendations
- HM Government, *Working Together to Safeguard Children 2026*
- Department for Education, *Information Sharing Duty: statutory guidance* (in force 30 September 2026)
- Department for Education, *National minimum standards for boarding schools* (2026)
- Samaritans, *Media Guidelines for Reporting Suicide* (2026)

Contacts, figures and references checked 28 September 2026. Verify at point of use.

Quick Reference Card

Immediate Danger Comes First

Immediate danger or possible risk to life: call 999 when it is needed, stay with the child, and summon the DSL or a deputy. Don't delay action to work through the steps. The 90-second Pause is for once immediate safety is secured, to stop an impulsive or investigative response.

Recognise

- Unexplained injuries, covering up, avoiding PE
- Withdrawal, low mood, poor sleep
- No disclosure needed, and no need to wait for a pattern

Respond

- Calm, private, kind. Never ask to see injuries
- First aid for fresh injuries; 999 if serious
- Ask directly about suicide
- TED, never "why". Don't promise secrecy

Notify

- DSL, the same day, even if the student denies it
- Suicidal thoughts, serious injury, anything taken: DSL now, not alone
- Life at risk: 999
- If in doubt, N before O

Document

- GOLD: Ground, Observe, Language, Do
- Plain description; their exact words
- No labels, no photographs, no method detail

Key Numbers

- Emergency: 999
- NHS 111: call 111 and choose the mental health option (option 2), for urgent mental health help in England, any time, or use NHS 111 online
- Samaritans: 116 123, free from any phone, any time, day or night, whatever your age
- Childline (for anyone under 19): 0800 1111, free and confidential; call any time, day or night, or use 1-2-1 chat at [childline.org.uk](https://www.childline.org.uk) (you may have to wait for a chat counsellor, so calling is usually quickest)
- Your DSL: named in the school's safeguarding policy

Online Evidence: One Rule

- **Ordinary messages or posts a student has already shown you:** note what is relevant (platform, date, who, exact words) in your record. Don't ask to see more or scroll through a phone, especially where messages may be sexual, because they may contain images. Don't log in to accounts, search profiles or investigate, and never transfer anything to a personal device.
- **Nude or semi-nude images of anyone under 18:** don't intentionally view, copy, screenshot, forward, save or delete them. Tell the DSL straight away. If you see one by accident, tell the DSL, ask for support if you need it, and record that it was seen and what you did, without describing the image (UKCIS).
- The DSL decides what happens to devices and content.

Your School's Contacts

This guide is generic. Schools add their own details here, or keep a completed copy with this card, so that anyone responding, including at night, has the routes to hand.

Contact	Name and how to reach them
Designated Safeguarding Lead (DSL)	
Deputy DSL(s)	
Out-of-hours and boarding on-call safeguarding contact	
How to record a concern (system or form)	
Local authority children's services front door	
Out-of-hours emergency duty team	
LADO (concerns about adults)	
Police: 999 (emergency); 101 (non-emergency)	

RESPOND Safeguarding Framework Reference Scenarios · Staff Guide 7

Scenario 7: Self-Harm Indicators

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Aligned with: KCSIE 2026 | Working Together to Safeguard Children 2026 | Information Sharing Duty (September 2026) | NICE NG225 | RSHE statutory guidance (published July 2025; statutory from September 2026) | NMS for Boarding Schools 2026 | Samaritans Media Guidelines

Last Updated: September 2026 | Next Review: September 2027, or on publication of KCSIE 2027 if sooner

For staff in schools and other settings working with children. You may share it with colleagues. Share the student and parent guides, not this page, with students and families.